

Heart Failure Self-Monitoring Log

A daily tracking tool to share with your healthcare team

This log is designed for people recently diagnosed with heart failure. Fill it out once a day, at roughly the same time, and bring it (or share it electronically) with your cardiologist, heart failure nurse, or primary care provider at each visit. Consistent daily tracking is one of the most effective ways to catch early warning signs before they become an emergency.

This resource is educational and does not replace advice from your own healthcare team. Always follow the specific instructions, targets, and thresholds your doctor or heart failure nurse gives you — they may differ from the general guidance below based on your individual condition.

When to Call Your Care Team vs. When to Seek Emergency Care

Call your heart failure team the same day if you notice:

- Weight gain of 2-3 lbs in one day, or about 5 lbs in a week
- Increasing swelling in your legs, ankles, or abdomen
- Increasing shortness of breath with activities that didn't used to bother you
- Needing more pillows to sleep, or waking up short of breath
- New or worsening fatigue that limits your normal activities
- A new irregular, fast, or unusually slow heartbeat
- Questions about a missed medication dose or side effects

Seek emergency care (call 911) if you experience:

- Severe shortness of breath at rest, or a feeling of suffocating
- Chest pain or pressure
- Fainting or feeling like you're about to pass out
- A very fast or irregular heartbeat with dizziness or chest discomfort
- Coughing up pink, frothy mucus

How to Use This Log

- Weigh yourself every morning after using the bathroom and before eating, on the same scale, wearing similar clothing.
- Check your blood pressure and pulse at roughly the same time each day, seated and rested for 5 minutes first.
- Fill in every row daily, even if the answer is 'no change' — trends matter more than any single day.
- Bring this log (or a photo/copy of it) to every appointment, and call your care team sooner if you notice any of the warning signs listed in the sections below.
- If you experience chest pain, severe shortness of breath at rest, fainting, or think you may be having a medical emergency, call 911 (or your local emergency number) — do not wait to log it first.

Comprehensive Daily Self-Assessment

Large print — print one fresh copy each day — 15 points covering vitals, symptoms, and daily care

Date: _____ Time: _____

For items 4-11, circle the single number from 1 to 10 that best matches how you feel right now. 1 always means the best/mildest end of the scale; 10 always means the worst. Report any chest discomfort or severe symptoms to your care team the same day.

1. WEIGHT

Same scale, same time, before breakfast, after the bathroom

Weight: _____ lbs

2. BLOOD PRESSURE

Seated, feet flat, rested 5 minutes

Top: _____ Bottom: _____

3. HEART RATE / PULSE

Count for a full 60 seconds if checking by hand

Heart rate: _____ beats per minute

4. SHORTNESS OF BREATH

Circle the number that best matches how you feel right now

1 2 3 4 5 6 7 8 9 10

1 = No shortness of breath

10 = Severe — can't catch your breath, even at rest

5. FATIGUE / ENERGY LEVEL

Circle one

1 2 3 4 5 6 7 8 9 10

1 = Normal energy

10 = Exhausted — can't get out of bed

6. SWELLING

Legs, ankles, feet, or belly — circle one

1 2 3 4 5 6 7 8 9 10

1 = No swelling

10 = Severe, widespread swelling

Where: _____

7. MOOD / MENTAL STATE

How are you feeling today, emotionally?

1 2 3 4 5 6 7 8 9 10

1 = Very good

10 = Very distressed, hopeless, or anxious

Notes: _____

8. DIZZINESS / LIGHTHEADEDNESS

Circle one

1	2	3	4	5	6	7	8	9	10
---	---	---	---	---	---	---	---	---	----

1 = None

10 = Severe — can't stand without help

Worse when standing up?

YES

NO

9. CHEST DISCOMFORT OR PRESSURE

Circle one — report any amount to your care team the same day

1	2	3	4	5	6	7	8	9	10
---	---	---	---	---	---	---	---	---	----

1 = None

10 = Severe — call 911 now

10. APPETITE

Circle one

1	2	3	4	5	6	7	8	9	10
---	---	---	---	---	---	---	---	---	----

1 = No appetite / not eating

10 = Normal appetite

Nausea today?

YES

NO

11. SLEEP QUALITY

Circle one

1	2	3	4	5	6	7	8	9	10
---	---	---	---	---	---	---	---	---	----

1 = Very poor

10 = Excellent

Hours slept: _____ Pillows used: _____ Woke up short of breath?

YES

NO

12. MEDICATIONS TAKEN AS PRESCRIBED

All medications taken today?

YES

NO

If no, which ones missed: _____

13. SODIUM / FLUID INTAKE

Approximate — doesn't need to be exact

Sodium today:

LOW

MODERATE

HIGH

Fluid intake today: _____ oz / cups

Stayed within my care team's limit?

YES

NO

14. PHYSICAL ACTIVITY

Check off everything you did today and jot down about how long

- ☐ Showering / bathing Minutes: _____
- ☐ Personal hygiene (dressing, grooming) Minutes: _____
- ☐ Walking around inside the house Minutes: _____
- ☐ Walk to the mailbox / short walk outside Minutes: _____
- ☐ Light household chores (cooking, laundry, tidying) Minutes: _____
- ☐ Heavier chores (vacuuming, yard work, moving items) Minutes: _____
- ☐ Animal / pet care (feeding, walking the dog) Minutes: _____
- ☐ Grocery shopping / errands Minutes: _____
- ☐ Climbing stairs Minutes: _____
- ☐ Cardiac rehab / physical therapy session Minutes: _____
- ☐ Driving Minutes: _____
- ☐ Other: _____ Minutes: _____

15. URINATION CHANGES

Compared to your usual pattern

MORE

NORMAL

LESS

Notes: _____

What Each Item Means and Why It Matters

The items below are ordered from most - to least critical based on how directly they signal early worsening of heart failure, according to current clinical guidelines and research. Every item is still worth tracking daily, this order simply reflects priority if you're deciding what to focus on first.

1. Daily body weight

Why it matters:

Sudden weight gain is usually fluid, not fat, and is often the earliest sign that heart failure is getting worse sometimes appearing days before you feel short of breath or swollen. Weighing yourself the same way every morning (after the bathroom, before breakfast, same scale, similar clothing) lets you and your care team catch fluid buildup early, before it sends you to the hospital.

Contact your care team if you notice:

- A gain of 2–3 lbs in one day
- A gain of about 5 lbs in a week
- Steady weight gain over several days even if each day's change seems small

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; American Association of Heart Failure Nurses (AAHFN). Daily Weights patient education sheet, aahfn.org

2. Blood pressure

Why it matters:

Many heart failure medications (like ACE inhibitors, ARNIs, and beta-blockers) change your blood pressure, and your care team uses your home readings to adjust doses safely. Blood pressure that runs too low can cause dizziness or falls; blood pressure that runs too high can strain the heart further.

Contact your care team if you notice:

- Readings consistently below 90/60 or your care team's personal low-end target, especially with dizziness or lightheadedness
- Readings consistently above the target your care team has given you
- A big change from your usual pattern in either direction

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; Lee S, et al. Effectiveness of a Smartphone App-Based Intervention With Bluetooth-Connected Monitoring Devices in Heart Failure (SMART-HF Trial). J Med Internet Res. 2024;26:e52075

3. Shortness of breath / breathing changes

Why it matters:

Fluid backing up into the lungs makes breathing harder — first with activity, then possibly at rest or while lying flat (needing extra pillows to sleep, or waking up gasping for air). Tracking this daily on a simple 0–3 scale helps your team see a trend, not just a bad day.

Contact your care team if you notice:

- Needing more pillows than usual to breathe comfortably at night
- Waking up suddenly short of breath
- Getting winded with activities that didn't used to bother you
- Shortness of breath at rest — this needs urgent attention

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; American Heart Association. Managing Heart Failure Symptoms. heart.org (reviewed 2024)

4. Swelling (edema) in legs, ankles, or belly

Why it matters:

Like weight gain, swelling is a visible sign of fluid retention. It commonly shows up in the lowest parts of the body (ankles, feet, lower legs) or the abdomen, and tends to get worse throughout the day.

Contact your care team if you notice:

- New or worsening swelling in your legs, ankles, or feet
- Your belly feeling more swollen or tight, or your clothes/shoes fitting tighter
- Swelling that leaves a dent when you press on it and doesn't bounce back quickly

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; American Heart Association. Managing Heart Failure Symptoms. heart.org (reviewed 2024)

5. Heart rate / pulse

Why it matters:

Your heart rate helps show how hard your heart is working and whether medications that slow the heart rate (like beta-blockers) are dosed correctly. A pulse that's unusually fast, slow, or irregular can be an early sign of a rhythm problem, which is common alongside heart failure.

Contact your care team if you notice:

- Resting heart rate consistently above 100 or below 50 beats per minute (unless your care team has told you a different target)
- A new irregular, fluttering, or racing heartbeat
- Heart rate changes paired with dizziness, chest discomfort, or fainting

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; Lee S, et al. Effectiveness of a Smartphone App-Based Intervention With Bluetooth-Connected Monitoring Devices in Heart Failure (SMART-HF Trial). J Med Internet Res. 2024;26:e52075

6. Fatigue and energy level

Why it matters:

Unusual tiredness or a drop in how much activity you can do without stopping to rest is one of the more subtle warning signs, because it can be mistaken for 'just getting older' or being out of shape. Rating it daily helps separate a normal tired day from a real downward trend.

Contact your care team if you notice:

- Feeling unusually exhausted doing tasks that used to be easy
- Needing to rest more often during normal daily activities
- A sudden, unexplained drop in energy over a few days

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; American Heart Association. Managing Heart Failure Symptoms. heart.org (reviewed 2024)

7. Mood and mental state

Why it matters:

Depression and anxiety are very common after a heart failure diagnosis — far more common than in the general population — and they make it harder to stick with medications, diet, and monitoring. Guidelines now recommend routinely screening for depression using simple questionnaires (PHQ-2, then PHQ-9 if needed), because treating mood problems improves both quality of life and a patient's ability to self-manage.

Contact your care team if you notice:

- Persistent sadness, hopelessness, or loss of interest in things you used to enjoy
- New or worsening anxiety, worry, or panic
- Trouble sleeping, concentrating, or wanting to withdraw from family and friends
- Any thoughts of self-harm — this needs immediate attention

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; Rutledge T, et al. Anxiety and Depression in Heart Failure: An Updated Review. Prog Cardiovasc Dis. 2023

8. Medication taken as prescribed

Why it matters:

Heart failure medications work best when taken consistently and exactly as directed — skipping doses (even unintentionally) is one of the most common causes of a preventable hospital readmission. A simple daily yes/no check helps you and your care team spot patterns, like missed doses on busy days.

Contact your care team if you notice:

- Missed doses, even occasional ones
- Side effects that make you want to stop or skip a medication on your own
- Confusion about which medications to take or when

Sources: Meraz R, et al. Resilience mediates the effects of self-care activation and hope on medication adherence in heart failure patients. Res Nurs Health. 2023;46(3):323-335; The use of technology to improve medication adherence in heart failure patients: a systematic review of RCTs. J Pharm Policy Pract. 2023; Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032

9. Sodium and fluid intake

Why it matters:

Extra salt makes the body hold onto more water, and heart failure already makes it harder to get rid of excess fluid — so what you eat and drink directly affects your weight, swelling, and breathing. Current expert guidance favors a moderate, consistent sodium intake (roughly at or under 2–3 grams per day, per your care team's specific target) over extremely strict restriction, which hasn't been shown to improve outcomes and can backfire.

Contact your care team if you notice:

- Eating notably saltier food than usual (restaurant meals, canned/processed foods, added salt)
- Drinking more fluid than your care team recommends, especially if you've been told to limit fluids
- Increased thirst alongside weight gain or swelling

Sources: Mullens W, et al. Dietary sodium and fluid intake in heart failure: a clinical consensus statement of the Heart Failure Association of the ESC. Eur J Heart Fail.

2024;26(4):730-740; Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. *Circulation*. 2022;145(18):e895-e1032

10. Physical activity level

Why it matters:

Regular, appropriately-paced physical activity (as approved by your care team, often through cardiac rehabilitation) improves heart failure symptoms and quality of life. Tracking what you did and how you tolerated it helps your team fine-tune your activity plan and catch a declining tolerance for exercise early.

Contact your care team if you notice:

- Being unable to do your usual activity or exercise without excess breathlessness or fatigue
- New chest discomfort, dizziness, or palpitations with activity
- A noticeable, sustained drop in how far or how long you can walk

Sources: Heidenreich PA, et al. 2022 AHA/ACC/HFSA Guideline for the Management of Heart Failure. Circulation. 2022;145(18):e895-e1032; The use of technology to improve medication adherence in heart failure patients: a systematic review of RCTs. J Pharm Policy Pract. 2023

My Medication List

Keep this page current and bring it to every appointment, ER visit, or hospital stay. Update it immediately whenever a medication, dose, or pharmacy changes.

My Information

Patient name: _____

Date of birth: _____

Known drug allergies / reactions: _____

My Care Team

Primary care provider: _____

Phone: _____

Cardiologist / heart failure specialist: _____

Phone: _____

My Pharmacy

Primary pharmacy name: _____

Address: _____

Phone: _____

Refill method (app, online portal, phone, auto-refill): _____

Backup pharmacy (if used): _____

My Medications (prescription, over-the-counter, vitamins, and supplements)

List every medication you take, including over-the-counter drugs, vitamins, and supplements — these can interact with heart failure medications too. Space is provided for up to 15 medications.

#	Medication (name & strength)	Dose & how taken	Purpose / reason	Prescribed by & start date	Refill due & qty	Notes / side effects
1						
2						
3						
4						
5						
6						
7						
8						

#	Medication (name & strength)	Dose & how taken	Purpose / reason	Prescribed by & start date	Refill due & qty	Notes / side effects
9						
10						
11						
12						
13						
14						
15						

Never stop, skip, or change a dose on your own — even if you feel fine or feel worse. Call your care team first, especially before starting any new over-the-counter medication or supplement.

Heart Failure Medications Glossary

Below are the medication classes most commonly prescribed for heart failure, listed by drug class with common generic and brand names. Not everyone is prescribed every medication — your combination depends on your type of heart failure, other health conditions, and how you respond to treatment. This list covers currently FDA-approved medications; it does not include drugs still in clinical trials that aren't yet approved for general use. Because heart failure is often preceded by another cardiac condition — coronary artery disease, high blood pressure, or an irregular heart rhythm — this list also includes a few common medications for those related conditions near the end, even though they don't treat heart failure directly. Never stop or change a dose without talking to your care team first.

This is general educational information, not a prescription or dosing guide. Always follow your own care team's specific instructions for your medications.

Drug class & common names	What it does	What to watch for / report
ACE inhibitors (lisinopril, enalapril, ramipril, captopril)	Relax blood vessels and reduce strain on the heart; one of the foundational 'four pillars' of heart failure treatment shown to improve survival.	Dry cough, dizziness, high potassium, rising creatinine (kidney marker), and rare but serious facial/throat swelling (angioedema) — seek emergency care for swelling of the face or throat.
ARBs (angiotensin receptor blockers) (losartan, valsartan, candesartan)	Work similarly to ACE inhibitors by relaxing blood vessels; often used instead of an ACE inhibitor if it causes a cough or isn't tolerated.	Dizziness, high potassium, rising creatinine.
ARNI (sacubitril/valsartan — Entresto)	A combination medication that's often preferred over ACE inhibitors/ARBs alone, shown to further reduce hospitalizations and improve survival.	Dizziness, low blood pressure, high potassium, rising creatinine; should not be combined with an ACE inhibitor.
Beta-blockers (carvedilol, metoprolol succinate, bisoprolol)	Slow the heart rate and reduce the heart's workload, helping it pump more efficiently and protecting it over time.	Fatigue, dizziness, very slow heart rate; do not stop abruptly without talking to your care team.
MRAs	A type of 'water pill' that	High potassium (needs regular

Drug class & common names	What it does	What to watch for / report
(mineralocorticoid receptor antagonists) (spironolactone, eplerenone)	also protects the heart from harmful long-term structural changes.	blood tests), and spironolactone can cause breast tenderness/enlargement in some patients.
Finerenone (Kerendia)	A newer, non-steroidal MRA (FDA-approved for heart failure in 2025) used for heart failure with mildly reduced or preserved ejection fraction (HFmrEF/HFpEF); shown to lower the risk of cardiovascular death and heart failure hospitalization.	High potassium (needs regular blood tests), similar to older MRAs, but generally causes fewer hormone-related side effects like breast tenderness.
SGLT2 inhibitors (dapagliflozin, empagliflozin)	Originally developed for diabetes; shown to reduce heart failure hospitalizations and support kidney function, regardless of diabetes status.	Genital or urinary tract infections, dehydration, and rarely a serious condition called diabetic ketoacidosis (even in people without diabetes) — report unusual nausea, vomiting, or abdominal pain.
Loop diuretics ('water pills') (furosemide/Lasix, bumetanide/Bumex, torsemide/Demadex)	Help the kidneys remove excess salt and water, relieving swelling and shortness of breath from fluid overload.	Dehydration, dizziness, low potassium or sodium, increased urination; your care team may ask you to adjust the dose based on your daily weight.
Thiazide-type diuretics (add-on) (metolazone)	Sometimes added to a loop diuretic for a short period when fluid buildup doesn't respond to the loop diuretic alone.	Significant drops in blood pressure, potassium, and sodium; typically used short-term under close supervision.
Digoxin (Lanoxin)	Helps the heart pump slightly more effectively and can help control a fast heart rate; used less often than in the past, mainly for symptom control.	Nausea, vision changes (halos, blurred/yellow vision), confusion, and irregular heartbeat — digoxin blood levels need periodic checking since too much can be toxic.
Ivabradine (Corlanor)	Slows the heart rate through a different pathway than beta-blockers, without	Visual brightness changes (phosphenes), slow heart rate, irregular heartbeat.

Drug class & common names	What it does	What to watch for / report
	lowering blood pressure; used when the heart rate stays high despite a beta-blocker.	
Hydralazine / isosorbide dinitrate (BiDil, or the two drugs given separately)	A combination of vasodilators that relax blood vessels, often added for patients (including many Black patients, per guideline recommendations) who need extra symptom relief or can't take an ACE inhibitor/ARB/ARNI.	Headache, low blood pressure, dizziness, especially when standing up quickly.
Vericiguat (Verquvo)	A newer medication that helps relax and protect the heart's blood vessels; used for patients with recent worsening heart failure despite other GDMT.	Low blood pressure and anemia; taken with food to reduce dizziness.
Aspirin & other antiplatelets (aspirin, clopidogrel, ticagrelor, prasugrel)	Not a routine heart failure medication — these prevent blood clots and are only prescribed if you also have coronary artery disease, a prior heart attack, or a coronary stent. Guidelines generally advise against routine aspirin use in heart failure without one of these specific reasons, since the bleeding risk can outweigh the benefit.	Increased bruising or bleeding, stomach upset; only take if your care team has given you a specific reason tied to your heart or blood vessels, not for heart failure alone.
Anticoagulants ('blood thinners') (warfarin, apixaban, rivaroxaban, dabigatran, edoxaban)	Not for heart failure itself, but often prescribed if you also have atrial fibrillation or a blood clot risk, to help prevent stroke.	Unusual bruising or bleeding, blood in urine or stool — seek care for any signs of serious bleeding.
Potassium and magnesium supplements	Sometimes prescribed alongside diuretics to	Take only as directed — too much potassium is also dangerous,

Drug class & common names	What it does	What to watch for / report
	replace minerals lost in the urine and help prevent dangerous heart rhythms.	especially if you're also on an ACE inhibitor, ARB, ARNI, or MRA.
IV iron (ferric carboxymaltose — Injectafer)	Given by IV infusion in a clinic for patients with HFrEF and iron deficiency (even without anemia); shown to improve energy, walking distance, and quality of life. Guidelines favor IV iron over oral iron pills, since oral iron is poorly absorbed and hasn't been shown to help in heart failure.	Temporary low blood phosphate, headache, nausea, and rare infusion reactions (flushing, dizziness) — usually monitored during the infusion visit.
Oral iron supplements (ferrous sulfate, ferrous gluconate)	Sometimes used for general iron-deficiency anemia unrelated to heart failure, but current heart failure guidelines do not recommend oral iron as an effective treatment for heart failure–related iron deficiency.	Constipation, dark stools, stomach upset; let your care team know if you're taking an over-the-counter iron supplement, since it may not be the right approach for your heart failure.
Tolvaptan (Samsca)	Used to correct low blood sodium (hyponatremia) that can occur with heart failure or its treatment; usually started in the hospital under close monitoring, not typically a long-term take-at-home pill.	Sodium levels must be corrected slowly and monitored closely — increased thirst and urination are expected; report any confusion or worsening symptoms right away.
IV inotropes (advanced/hospital-based therapy) (dobutamine, milrinone)	Reserved for advanced, hospital-based heart failure to temporarily strengthen the heart's pumping when pill-based medications aren't enough on their own; given continuously through an IV, sometimes at home	Irregular heartbeat, low blood pressure; these require close medical supervision and are not part of routine at-home pill regimens.

Drug class & common names	What it does	What to watch for / report
	with a pump for patients awaiting transplant or receiving palliative/comfort-focused care.	
Statins (atorvastatin, rosuvastatin, simvastatin)	Not a heart failure medication itself — these lower cholesterol and treat the coronary artery disease that's often the underlying cause or a major contributor to heart failure. Many people with heart failure take one because of a prior heart attack, stent, or bypass surgery.	Muscle aches or weakness, and rarely liver enzyme changes — report unexplained or severe muscle pain to your care team.
Calcium channel blockers (amlodipine, felodipine; verapamil/diltiazem used with caution)	Not typically used to treat heart failure with reduced ejection fraction — some types (verapamil, diltiazem) can actually make HFrEF worse. But 'dihydropyridine' types like amlodipine are sometimes prescribed for blood pressure or chest pain (angina) from coronary artery disease that preceded your heart failure.	Swelling in the ankles or feet, dizziness, flushing; make sure your care team knows your full heart failure type before starting one of these for another condition.
Antiarrhythmic medications (amiodarone)	Not for heart failure itself, but sometimes prescribed if you also have an irregular heart rhythm — such as atrial fibrillation or a ventricular arrhythmia — that developed alongside or before your heart failure.	Amiodarone needs regular monitoring of your thyroid, liver, and lungs; report any new cough, shortness of breath, or vision changes to your care team.

Glossary of Heart Failure Terms

Plain-English definitions of medical terms you're likely to hear from your care team, read in your discharge paperwork, or see in this log.

Term	Plain-English meaning
Heart failure (HF)	A condition where the heart muscle doesn't pump blood as well as it should, causing fluid to build up in the lungs, legs, or abdomen and leaving the body without enough oxygen-rich blood.
Congestive heart failure (CHF)	An older term for heart failure, still used interchangeably by many doctors and hospitals; it emphasizes the fluid ('congestion') buildup that heart failure causes.
Cardiomyopathy	Disease of the heart muscle itself that makes it weaker, stiffer, or abnormally shaped, and is a common underlying cause of heart failure.
Ischemic cardiomyopathy	Heart muscle damage caused by reduced blood flow, usually from coronary artery disease or a past heart attack.
Non-ischemic cardiomyopathy	Heart muscle damage from a cause other than blocked arteries, such as viral infection, genetics, chemotherapy, alcohol, or long-term high blood pressure.
Ejection fraction (EF)	A percentage measuring how much blood the left side of your heart pumps out with each beat. Used to classify heart failure as reduced (HFrEF), mildly reduced, or preserved (HFpEF) EF.
HFrEF	Heart failure with reduced ejection fraction — the heart's main pumping chamber squeezes weakly (EF $\leq$ 40%).
HFmrEF	Heart failure with mildly reduced ejection fraction (EF 41-49%) — a middle category between HFrEF and HFpEF.
HFpEF	Heart failure with preserved ejection fraction — the heart squeezes normally but doesn't relax and fill properly (EF $\geq$ 50%).
Systolic dysfunction	A problem with the heart's squeezing (pumping) ability — the hallmark of HFrEF.
Diastolic dysfunction	A problem with the heart's ability to relax and fill with blood between beats — the hallmark of HFpEF.
Preload	The amount of blood stretching the heart's chambers just before it contracts; affected by how much fluid is in the body.
Afterload	The resistance the heart must pump against to push blood out to the body, largely driven by blood pressure and blood vessel stiffness.
Cardiac output	The amount of blood the heart pumps per minute; a key measure of how well the heart is meeting the body's needs.
Stroke volume	The amount of blood pumped out of the heart with a single beat.
Edema	Swelling caused by fluid building up in body tissue, commonly

Term	Plain-English meaning
	seen in the legs, ankles, feet, or abdomen in heart failure.
Pitting edema	Swelling that leaves a temporary dent when pressed with a finger — a classic sign of fluid retention.
Anasarca	Severe, widespread swelling throughout the body, seen in advanced or poorly controlled heart failure.
Ascites	Fluid buildup in the abdominal cavity, which can cause belly swelling, bloating, or tight-fitting clothes.
Pulmonary edema	Fluid buildup in the lungs that causes severe shortness of breath and can be a medical emergency.
Congestion	A medical term for fluid buildup in the lungs or body caused by the heart not pumping efficiently; this is what causes swelling and breathlessness.
Euvolemic	A state of normal, balanced fluid levels in the body — the goal of heart failure treatment.
Hypervolemia	Having too much fluid in the body, which worsens heart failure symptoms like swelling and breathlessness.
Orthopnea	Shortness of breath that happens when lying flat, usually relieved by sitting up or using extra pillows.
Paroxysmal nocturnal dyspnea (PND)	Sudden, severe shortness of breath that wakes you from sleep, caused by fluid shifting into the lungs while lying down.
Dyspnea	The medical term for shortness of breath or difficulty breathing.
Rales / crackles	Abnormal crackling sounds heard in the lungs with a stethoscope, often caused by fluid; something your care team listens for at visits.
Jugular venous distension (JVD)	Visible bulging of the neck veins that can indicate too much fluid and pressure in the heart and blood vessels.
S3 gallop	An extra, abnormal heart sound your doctor may hear with a stethoscope that can suggest fluid overload or a weakened heart.
Cachexia	Unintentional weight loss and muscle wasting that can occur in advanced or long-standing heart failure.
Hepatomegaly	An enlarged liver, which can happen when heart failure causes blood to back up into the liver.
Arrhythmia	Any abnormal heart rhythm — too fast, too slow, or irregular.
Atrial fibrillation (AFib)	A common irregular, often rapid heart rhythm that frequently occurs alongside heart failure and raises stroke risk.
Tachycardia	A heart rate that is faster than normal (generally over 100 beats per minute at rest).
Bradycardia	A heart rate that is slower than normal (generally under 60 beats per minute), which can sometimes be caused by heart failure

Term	Plain-English meaning
	medications.
Hypotension	Low blood pressure, which can cause dizziness or fainting and is a common side effect of several heart failure medications.
Hypertension	High blood pressure, a major risk factor for developing and worsening heart failure.
Hyponatremia	Low blood sodium levels, which can occur in heart failure and sometimes requires fluid restriction to correct.
Hyperkalemia	High blood potassium levels — a possible side effect of some heart failure medications (ACE inhibitors, ARBs, ARNI, MRAs) that requires monitoring with blood tests.
Renal function / kidney function	How well the kidneys are filtering waste from the blood; closely linked to heart failure and monitored regularly because many HF medications affect the kidneys.
GFR (Glomerular Filtration Rate)	A blood-test-based estimate of how well your kidneys are working, tracked over time in heart failure care.
Creatinine	A waste product measured in blood tests to estimate kidney function; levels can shift with heart failure medications and need monitoring.
BNP / NT-proBNP	A blood test measuring a hormone released when the heart is under strain; higher levels can indicate worsening heart failure.
Anemia	A lower-than-normal red blood cell count, common in heart failure patients and a cause of extra fatigue.
Sleep apnea	A sleep disorder involving pauses in breathing during sleep that is common in heart failure and can worsen symptoms if untreated.
Echocardiogram (echo)	An ultrasound of the heart used to measure ejection fraction and check heart structure and valve function.
Electrocardiogram (EKG/ECG)	A quick test that records the heart's electrical activity to check heart rhythm and detect certain problems.
Cardiac catheterization	A procedure where a thin tube is guided into the heart's blood vessels or chambers to check for blockages, measure pressures, or take tissue samples.
Stress test	A test that checks how the heart performs under physical exertion (or medication that mimics exertion), often used to look for blocked arteries.
Six-minute walk test	A simple test measuring how far you can walk in six minutes, used to track functional capacity and treatment response over time.
Chest X-ray	An imaging test that can show fluid in the lungs, heart size, and other changes related to heart failure.
Cardiac MRI	A detailed imaging scan of the heart sometimes used to identify

Term	Plain-English meaning
	the specific cause of cardiomyopathy.
Valve regurgitation	A heart valve that doesn't close tightly, allowing blood to leak backward; can contribute to or worsen heart failure.
Valve stenosis	A heart valve that has narrowed or stiffened, restricting blood flow and increasing strain on the heart.
NYHA Class (I-IV)	A scale (New York Heart Association) describing how much your symptoms limit physical activity, from Class I (no limitation) to Class IV (symptoms at rest).
ACC/AHA Stage (A-D)	A staging system describing heart failure progression, from Stage A (at risk, no disease yet) to Stage D (advanced, treatment-resistant disease).
GDMT (Guideline-Directed Medical Therapy)	The combination of evidence-based medications (such as ACE inhibitors/ARNI, beta-blockers, MRAs, and SGLT2 inhibitors) recommended for most heart failure patients to improve survival and reduce hospitalizations.
Titration	The gradual, step-by-step adjustment of a medication dose (often upward) to reach the most effective and best-tolerated level.
Decompensation	A sudden worsening of heart failure symptoms, often due to fluid overload, that may require urgent treatment or hospitalization.
Comorbidity	Another health condition (such as diabetes, kidney disease, or high blood pressure) that occurs alongside heart failure and can affect its management.
Palpitations	A sensation of a racing, pounding, fluttering, or irregular heartbeat.
Polypharmacy	Taking multiple medications at once, which is common in heart failure care and requires extra attention to interactions and side effects.
Angioedema	Rare but serious swelling of the face, lips, tongue, or throat that can occur with ACE inhibitors and requires immediate medical attention.
ACE inhibitor cough	A dry, persistent cough that some people develop on ACE inhibitors, which usually resolves after switching to a different medication class (such as an ARB).
Cardiac rehabilitation ('cardiac rehab')	A supervised program of exercise, education, and support for people with heart conditions, shown to improve symptoms and quality of life.
Implantable cardioverter-defibrillator (ICD)	A small device placed under the skin that monitors heart rhythm and delivers a shock to correct dangerous, life-threatening rhythms.
Cardiac	A specialized pacemaker that helps the heart's chambers beat in a

Term	Plain-English meaning
resynchronization therapy (CRT) / biventricular pacemaker	more coordinated way, used in certain types of heart failure.
Left ventricular assist device (LVAD)	A mechanical pump implanted to help a severely weakened heart circulate blood, used for advanced heart failure.
Heart transplant	Surgery to replace a severely failing heart with a healthy donor heart, reserved for the most advanced cases.
Telehealth / remote monitoring	Using phone, video, or connected devices (like Bluetooth scales or blood pressure cuffs) to share health data with your care team without an in-person visit.
Fluid restriction	A care-team-directed limit on how much liquid you drink per day, sometimes used to help control fluid buildup.
Sodium restriction	Limiting dietary salt intake to help the body avoid holding onto extra fluid.
DASH diet	'Dietary Approaches to Stop Hypertension' — an eating pattern rich in fruits, vegetables, and low-fat dairy that's often recommended alongside sodium limits.
PHQ-2 / PHQ-9	Short standardized questionnaires used to screen for depression; PHQ-2 is a quick 2-question screen, and PHQ-9 is a more detailed 9-question follow-up.
Self-care/self-management	The everyday actions patients take — like monitoring symptoms, taking medications, and adjusting diet or activity — to manage their own condition between medical visits.
Advance directive	A legal document stating your wishes for medical care if you're unable to speak for yourself, often discussed as part of heart failure planning.
Palliative care	Specialized medical care focused on relieving symptoms and improving quality of life at any stage of a serious illness — not the same as end-of-life care.
Hospice care	Care focused on comfort rather than cure, typically for patients with advanced illness and a limited life expectancy.

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